

Wardle Editorial Canon

Standards for Governed Editorial Artifacts

Part II — Diagnosis Governance

1. Purpose

Diagnosis Governance defines how a diagnosis enters, develops within, and remains governed by the Wardle Editorial Operating System.

The diagnosis is the central organising entity of WEOS.

Cases, education, teaching rules, reasoning paths, differentials, evidence relationships, graph artifacts, and publication decisions all depend upon a stable diagnosis identity.

Diagnosis Governance therefore establishes the foundation upon which every other editorial capability operates.

Its purpose is to ensure that each diagnosis is:

- clinically identifiable
- terminologically consistent
- appropriately classified
- educationally scoped
- suitable for intended Wardle use
- linked to its governed editorial artifacts
- developed through an explicit onboarding process
- protected from duplication or ambiguous identity
- continuously maintainable

Diagnosis Governance does not require every diagnosis to possess complete educational content at the moment of registration.

It requires that every diagnosis possess a trustworthy identity and a clear path toward editorial development.

2. Diagnosis as the Primary Governance Entity

A diagnosis within WEOS is not merely a label attached to cases.

It is a first-class governed clinical knowledge entity.

The diagnosis provides the enduring identity around which the editorial system organises:

- terminology
- taxonomy
- aliases
- cases
- diagnosis education
- teaching rules
- learning goals
- reasoning paths
- differential relationships
- graph candidates and facts
- evidence relationships
- AI draft history
- publication readiness
- maintenance activity

Cases may change.

Education may be revised.

Teaching relationships may be added or deprecated.

The diagnosis remains the stable conceptual centre of this knowledge.

A diagnosis should therefore possess an identity independent of any single case, title, explanation, or educational section.

3. Diagnosis Registry Entry

3.1 Purpose

The Diagnosis Registry Entry is the authoritative identity record for a diagnosis within WEOS.

It establishes which clinical concept Wardle recognises as a distinct diagnosis and provides the stable reference through which related editorial artifacts are connected.

The registry is not merely a search dictionary.

It is the identity and taxonomy foundation of the editorial system.

3.2 Canonical responsibilities

A Diagnosis Registry Entry should establish:

- canonical diagnosis identity
- preferred learner-facing label
- normalised identity for matching and deduplication
- accepted aliases
- classification and taxonomy
- editorial development status
- operational availability
- suitability for gameplay
- suitability for automated generation
- generation guidance
- governance history

The registry should answer:

- What diagnosis is this?
 - Is it distinct from other registry entries?
 - How should it be displayed?
 - How may users or systems refer to it?
 - Which specialty and clinical context does it belong to?
 - May it appear as a Wardle answer?
 - May new cases be generated for it?
 - Has its diagnosis-level editorial foundation been developed?
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3.3 Identity standard

Every registry diagnosis must possess:

- a stable identifier
- a canonical clinical name
- a normalised canonical identity
- a display label
- a registry status
- an active or inactive operational state

The canonical name should represent the preferred clinical diagnosis rather than a descriptive case title.

Examples of acceptable canonical identities include:

- Chronic Pulmonary Aspergillosis
- Unstable Angina
- Neurogenic Bladder
- Scaphoid Fracture

Examples of unsuitable canonical identities include:

- Chest Pain in a Middle-Aged Man
- Progressive Fatigue with Hyperpigmentation
- Swollen Painful Wrist Following a Fall
- Fever, Murmur, and Splinter Haemorrhages

Descriptive phrases may be appropriate as case titles.

They should not replace diagnosis identity.

3.4 Canonical and display identity

The canonical diagnosis name and learner-facing display label may differ where clinically justified.

The canonical name exists for governance and identity.

The display label exists for clear presentation.

Differences should be deliberate rather than accidental.

The display label must not:

- introduce diagnostic ambiguity
 - use non-standard terminology without justification
 - conceal the true diagnosis behind a narrative phrase
 - conflict with accepted aliases
 - imply a subtype that the registry entry does not represent
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3.5 Normalisation

Every diagnosis should possess a normalised identity suitable for:

- duplicate detection
- alias resolution
- case mapping
- differential mapping
- search
- migration
- graph linking

Normalisation should remove superficial variation without combining clinically distinct diagnoses.

Normalisation may address:

- letter case
- punctuation
- spacing
- common spelling variants
- accepted abbreviation formats

Normalisation must not erase meaningful clinical distinctions.

For example:

- Type 1 Diabetes Mellitus and Type 2 Diabetes Mellitus must remain distinct.
 - Crohn Disease and Ulcerative Colitis must remain distinct.
 - Stable Angina and Unstable Angina must remain distinct.
 - Acute and chronic forms should not be merged where they carry different educational meaning.
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4. Diagnosis Aliases

4.1 Purpose

Aliases allow multiple clinically acceptable terms to resolve to one governed diagnosis identity.

Aliases support:

- learner answer matching
- case mapping
- search
- imported content
- terminology variation
- abbreviation handling
- legacy migration

Aliases do not create additional diagnoses.

They provide alternative paths to the same diagnosis.

4.2 Alias categories

The Canon recognises the following alias purposes.

Canonical

The authoritative diagnosis name.

Accepted

A clinically valid alternative term that may be accepted in search or learner answers.

Abbreviation

A recognised shortened form.

Search Only

A term useful for discovery or legacy matching but unsuitable as a preferred learner-facing answer.

4.3 Alias quality requirements

An alias should be:

- clinically equivalent to the canonical diagnosis
- unambiguous within the intended context
- appropriately classified
- free from unnecessary descriptive phrasing
- traceable to the correct registry entry

An alias must not be used to hide distinct clinical concepts within one diagnosis record.

Where two terms overlap but are not truly equivalent, they should be represented through relationships rather than aliases.

4.4 Alias governance

Alias creation should be reviewed where the term could affect:

- learner answer acceptance
- differential resolution
- graph identity
- duplicate detection
- imported content mapping

Aliases that are misleading, outdated, overly broad, or clinically unsafe should be rejected or classified as search-only.

5. Diagnosis Registry Status

The diagnosis registry lifecycle describes the standing of the diagnosis identity itself.

It does not describe the completeness of its cases or education.

5.1 Draft

The registry identity is being prepared or reviewed and should not yet participate fully in operational use.

5.2 Active

The diagnosis is recognised as a valid registry entity.

Active status does not automatically mean:

- editorial development is complete
- cases exist
- cases are playable
- education is published
- automated generation is safe

5.3 Hidden

The diagnosis remains in the registry but should not appear in ordinary discovery or selection contexts.

Hidden status may be appropriate for:

- internal mapping records
- incomplete migration targets
- restricted editorial concepts
- temporarily suppressed diagnoses

5.4 Deprecated

The registry identity should no longer be used for new editorial work.

A deprecated diagnosis may be preserved because of:

- historical cases
- legacy mappings
- audit requirements
- replacement by another registry diagnosis
- terminology evolution

Deprecation should identify the preferred replacement where one exists.

6. Operational Availability

Registry status and operational availability must remain separate.

6.1 Active flag

The active flag determines whether the diagnosis currently participates in operational registry use.

An active registry identity should not be assumed to possess complete educational governance.

6.2 Playability

Playability determines whether the diagnosis may appear as the correct answer in a learner-facing case.

A diagnosis may be non-playable because it is:

- overly broad
- primarily an alias or mapping concept
- educationally unsuitable as a final answer
- reserved for future development
- insufficiently specific
- not intended for gameplay

Playability is a diagnosis-level permission.

It does not establish that any particular case is playable.

Both the diagnosis and the case must independently satisfy their playability requirements.

6.3 Generatability

Generatability determines whether automated systems are permitted to create new case drafts for the diagnosis.

A diagnosis may be playable but not generatable.

This distinction is appropriate where:

- cases require specialist nuance
- automated generation creates a high risk of ambiguity
- the diagnosis is clinically heterogeneous

- evidence or reasoning foundations are incomplete
- generation policies prohibit automated authoring

Generatability permits draft production.

It does not permit publication.

7. Diagnosis Metadata

7.1 Purpose

Diagnosis metadata establishes the clinical, educational, and operational context of a registry diagnosis.

Metadata supports:

- curriculum organisation
- specialty ownership
- case balancing
- generation guidance
- difficulty sequencing
- search
- coverage analysis
- prioritisation
- portfolio governance

Metadata should describe the diagnosis itself rather than the characteristics of one particular case.

7.2 Core classification metadata

Diagnosis metadata may include:

- category
- specialty
- subspecialty
- body system
- organ system
- difficulty band
- rarity band
- typical clinical setting
- relevant age group
- urgency level
- ICD coding
- descriptive or compositional status
- search priority

7.3 Specialty

Specialty identifies the broad clinical discipline responsible for the diagnosis.

Examples include:

- Cardiology
- Neurology
- Endocrinology
- Respiratory Medicine
- Orthopaedics
- General Surgery
- Paediatrics
- Psychiatry

Specialty should support editorial ownership and curriculum grouping.

It should not be used as a substitute for body system or category.

7.4 Subspecialty

Subspecialty provides a more specific clinical grouping within the specialty.

Examples include:

- Electrophysiology
- Heart Failure
- Neuromuscular Medicine
- Movement Disorders
- Metabolic Bone Disease
- Urogynaecology

Subspecialty should only be used where it adds meaningful editorial organisation.

7.5 Category

Category describes the educational or disease grouping of the diagnosis.

Examples include:

- Infectious
- Autoimmune
- Metabolic

- Vascular
- Neoplastic
- Degenerative
- Traumatic

Category should support knowledge organisation across specialties.

7.6 Body and organ systems

Body system describes the broad physiological or anatomical system.

Organ system provides a more granular organ or anatomical focus where useful.

These fields should not duplicate specialty without adding meaning.

For example:

- Specialty: Neurology
 - Body system: Nervous System
 - Organ system: Peripheral Nerve
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7.7 Difficulty band

Difficulty band describes the expected educational complexity of the diagnosis.

The current canonical bands are:

- Basic
- Intermediate
- Advanced

Difficulty should consider:

- complexity of recognition
- breadth of differentials
- reasoning depth
- required prior knowledge
- subtlety of discriminators
- management complexity

Difficulty should not be based solely on disease rarity.

7.8 Rarity band

Rarity supports balanced curriculum and case planning.

The current canonical bands are:

- Common
- Uncommon
- Rare

Rarity should not determine educational importance by itself.

Rare diagnoses may be essential where they are:

- clinically dangerous
 - frequently confused
 - important examination topics
 - necessary exclusions
 - central to escalation reasoning
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7.9 Clinical setting

Clinical setting describes where the diagnosis is typically encountered or most appropriately taught.

Current categories include:

- Outpatient
- Emergency
- Inpatient
- Intensive Care
- Community

This metadata should guide case framing without preventing alternative presentations.

7.10 Age group

Age group identifies the population in which the diagnosis is educationally most relevant.

Current categories include:

- Paediatric
- Adult
- Geriatric
- Any

Age-group metadata should not prohibit clinically valid atypical cases.

Such cases should, however, be intentionally authored.

7.11 Urgency

Urgency describes the usual clinical priority associated with recognising the diagnosis.

Current categories include:

- Routine
- Urgent
- Emergent

Urgency should influence:

- case framing
 - escalation teaching
 - management emphasis
 - prioritisation of review
 - learner-safety requirements
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8. Generation Guidance

8.1 Purpose

Diagnosis-level generation guidance constrains how new cases and educational content should be drafted.

It provides reusable diagnosis-specific direction rather than relying on each generation request to rediscover the clinical structure.

Generation guidance may include:

- preferred clue types
- excluded clue types
- difficulty guidance
- presentation constraints
- required discriminators
- required mimics
- common errors to avoid
- appropriate settings
- age guidance
- urgency guidance

8.2 Preferred clue types

Preferred clue types identify information domains that are educationally central to diagnosing the condition.

Examples include:

- history
- physical examination
- laboratory testing
- imaging
- pathology
- risk factors
- clinical course

Preferred clues guide generation.

They should not require every case to use the same sequence.

8.3 Excluded clue types

Excluded clue types identify information that should be avoided or de-emphasised.

This may be appropriate where a clue type:

- reveals the diagnosis too early
- is not central to clinical reasoning
- creates dependence on specialist testing
- produces repetitive cases
- does not suit the intended learner level

Exclusion should be educationally justified.

It should not merely reflect absent data in existing cases.

9. Diagnosis Editorial Onboarding

9.1 Purpose

Diagnosis Editorial Onboarding represents the development of a registry diagnosis from an identity record into an editorially usable diagnosis foundation.

Onboarding is not publication.

It is not the lifecycle of any single artifact.

It is the coordinated progress of diagnosis-level editorial development.

9.2 Onboarding stages

New

The diagnosis has entered the registry but diagnosis-level editorial development has not meaningfully begun.

Rules Started

Teaching rules have begun to define the diagnosis's educational constraints and priorities.

Brief Started

The editorial brief has begun to consolidate learning goals, required rules, mimics, pitfalls, investigations, management anchors, and generation guidance.

Education Started

Diagnosis education has entered drafting or generation.

Case Started

At least one clinical case has entered authoring or generation.

Ready for Review

The diagnosis possesses sufficient scaffold and content for coordinated diagnosis-level review.

Complete

The required onboarding components have satisfied the defined completion policy.

Completion does not necessarily mean that:

- all future cases have been written
- all graph work is finished
- maintenance is no longer required
- every artifact is published

It means the diagnosis has completed its initial editorial foundation.

9.3 Onboarding principles

Onboarding should:

- reveal missing foundational work
- guide the next appropriate editorial action
- distinguish absence from failure
- avoid treating generated drafts as approved artifacts
- remain explainable
- preserve independent artifact lifecycles
- support iterative progress

Onboarding should not become a second publication lifecycle.

10. Diagnosis Editorial Brief

10.1 Purpose

The Diagnosis Editorial Brief is the diagnosis-level specification that translates registry identity into educational intent.

It is the editorial contract for future case generation, education development, reasoning design, and graph development.

The brief answers:

- What must learners understand?
 - Which teaching rules are required?
 - Which mimics must be addressed?
 - Which pitfalls must be prevented?
 - Which investigations are educationally important?
 - Which management anchors must be taught?
 - What difficulty is appropriate?
 - How should cases be generated?
 - How should education be structured?
 - What graph development is needed?
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10.2 Relationship to other artifacts

The brief should guide:

- teaching-rule selection
- case generation
- learning-goal coverage
- diagnosis education
- reasoning paths
- differential relationships
- graph candidates
- publication assessment

The brief does not replace these artifacts.

It coordinates them.

10.3 Summary

The brief summary should define the diagnosis's core editorial identity.

It should communicate:

- what makes the diagnosis educationally important
- the central recognition pattern
- the primary reasoning challenge
- major learner risks
- the intended scope of Wardle coverage

The summary should not duplicate the learner-facing diagnosis education summary.

It is written for editorial use.

10.4 Learning goals

Learning goals define what the learner should be able to recognise, distinguish, interpret, or act upon.

High-quality learning goals should be:

- diagnosis-specific
- clinically meaningful
- assessable through cases or recall
- linked to reasoning rather than trivia
- appropriately scoped

- capable of coverage analysis

Examples include:

- Recognise the pattern of chronic pulmonary symptoms with cavitary lung disease and fungal evidence.
- Distinguish unstable angina from myocardial infarction using symptoms, biomarkers, and electrocardiography.
- Identify neurogenic bladder in a patient with neurological dysfunction and abnormal voiding.

Learning goals should not be vague statements such as:

- Understand the diagnosis.
 - Know the symptoms.
 - Learn management.
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10.5 Required teaching rules

The brief should identify the teaching rules that must govern downstream content.

A required teaching rule may constrain:

- acceptable presentations
- required differentials
- expected evidence
- clue timing
- educational emphasis
- case generation
- graph development

A brief should reference governed teaching rules rather than duplicating their full content.

10.6 Required mimics

Required mimics define diagnoses that should be considered in the diagnosis's educational framework.

A mimic may be required because it is:

- commonly confused with the diagnosis
- clinically dangerous to miss
- central to diagnostic discrimination
- important for examinations
- necessary for realistic case reasoning

Required mimic lists should not become indiscriminate differential catalogues.

They should prioritise educational value.

10.7 Required pitfalls

Required pitfalls identify learner errors the diagnosis must explicitly guard against.

Examples include:

- anchoring on a common diagnosis despite contradictory evidence
- over-relying on a single investigation
- confusing a complication with the primary diagnosis
- interpreting a normal early test as exclusion
- missing an atypical presentation
- exposing the answer through an overly specific early clue

Pitfalls in the editorial brief define educational requirements.

Learner-facing pitfalls are addressed separately within Diagnosis Education.

10.8 Investigation anchors

Investigation anchors define the diagnostic tests or findings that are central to teaching the diagnosis.

They may identify:

- first-line investigations
- confirmatory investigations
- characteristic findings
- important limitations
- tests that distinguish mimics
- resource-limited alternatives

Investigation anchors guide cases and education.

They should not become complete clinical protocols.

10.9 Management anchors

Management anchors define the management principles that Wardle should consistently teach.

They may include:

- immediate stabilisation priorities
- first-line treatment
- definitive management
- escalation thresholds
- contraindicated actions
- specialist referral
- prevention of complications

Management anchors should remain appropriate to the educational scope of Wardle and the target learner level.

10.10 Difficulty guidance

Difficulty guidance explains how the diagnosis should be adapted across learner levels.

It may define:

- which presentations suit basic learners
- which mimics suit intermediate cases
- which atypical patterns suit advanced cases
- how clue specificity should change
- when specialist investigations are acceptable
- how much ambiguity is educationally appropriate

Difficulty guidance should produce different reasoning challenges rather than merely more obscure facts.

10.11 Case-generation guidance

Case-generation guidance defines the boundaries within which cases should be authored.

It may describe:

- preferred presentation
- acceptable clinical settings
- required reasoning objective
- clue sequencing expectations
- prohibited early disclosures
- required mimics
- expected discriminator timing
- management or escalation focus
- desired case diversity

Generation guidance should enable variation without losing educational identity.

10.12 Education guidance

Education guidance defines the expected emphasis of learner-facing diagnosis education.

It may identify:

- sections requiring particular depth
- high-risk misconceptions
- required differentials
- essential investigations
- management priorities
- scoring systems
- examination pearls
- recall themes

Education guidance should improve coherence across independently generated sections.

10.13 Graph guidance

Graph guidance defines which relationships should be represented or explored.

It may identify:

- priority mimics
- shared presentations
- discriminator relationships
- evidence relationships
- complication relationships
- management contrasts
- recall relationships

Graph guidance should direct candidate generation.

It should not automatically establish graph facts.

11. Teaching Rules

11.1 Purpose

A Diagnosis Teaching Rule is a governed diagnosis-specific instruction that constrains or guides content generation, review, education, reasoning, or graph development.

Teaching rules convert clinical and educational judgement into reusable editorial policy.

They exist to prevent the editorial system from repeatedly rediscovering the same requirements.

11.2 Teaching-rule structure

A teaching rule should possess:

- stable identity
 - title
 - category
 - importance
 - rationale
 - acceptable manifestations
 - required differentials
 - expected evidence
 - difficulty guidance
 - timing constraints
 - applicability scope
 - lifecycle state
 - source
 - version
-

11.3 Stable key

A teaching rule should possess a stable key independent of its wording.

This allows the rule to be:

- revised
- referenced by the editorial brief
- linked to evidence
- linked to teaching relationships
- used during generation
- used during validation
- measured for coverage

Changing the title should not break the rule's identity.

11.4 Category

Teaching-rule categories should represent clinically meaningful editorial concerns.

Possible categories include:

- presentation
- discriminator
- differential
- investigation
- management
- complication
- escalation
- clue progression
- learner misconception
- generation constraint

Categories should support governance and analysis rather than visual organisation alone.

11.5 Importance

Importance identifies the consequence of failing to satisfy the rule.

A rule may be:

- essential
- high priority
- recommended
- contextual

Essential rules may block approval or publication where applicable.

Importance should not be inferred solely from display order.

11.6 Rationale

Every teaching rule should explain why it exists.

The rationale should identify:

- the clinical risk
- the educational objective
- the common error being prevented
- the reasoning value
- the governance consequence

A rule without rationale becomes difficult to review, maintain, or safely apply.

11.7 Acceptable manifestations

Acceptable manifestations describe clinically valid ways the rule may appear in content.

They provide controlled flexibility.

For example, a teaching rule about heart failure presentation may permit:

- exertional dyspnoea
- orthopnoea
- paroxysmal nocturnal dyspnoea
- peripheral oedema

The rule should not require every manifestation in every case.

11.8 Required differentials

A teaching rule may identify differentials that must be represented or considered.

This may affect:

- case explanation
- clue progression
- reasoning paths
- education
- graph development

Required differentials should resolve to registry diagnoses where possible.

11.9 Expected evidence

Expected evidence identifies findings that should support or discriminate the diagnosis.

This may include:

- symptoms
- signs
- laboratory findings
- imaging
- risk factors
- clinical course
- management response

Expected evidence can guide both generation and validation.

11.10 Avoid-too-early constraint

Some findings are diagnostically valuable but educationally harmful when introduced too early.

The avoid-too-early constraint should be used where a clue:

- reveals the diagnosis prematurely
- collapses meaningful differential reasoning
- prevents progressive uncertainty
- converts a reasoning case into recall of a pathognomonic phrase

This constraint should influence clue sequencing rather than exclude the evidence entirely.

11.11 Applicability

A teaching rule may apply to:

- diagnosis education
- case generation
- graph development
- more than one of these domains

Applicability must be explicit.

A rule intended for case generation should not automatically block education publication unless policy states that it applies there.

11.12 Teaching-rule lifecycle

Teaching rules should support at least the following conceptual states:

- proposed
- awaiting review
- active
- rejected
- deprecated

An active teaching rule is available to influence editorial work.

Deprecation should preserve history and identify replacement where applicable.

12. Learning Goals

12.1 Canonical status

Learning goals currently exist within the Diagnosis Editorial Brief and are measured through case learning-goal coverage.

They should therefore be treated as governed diagnosis-level educational requirements even where they are stored as structured brief content rather than separate first-class records.

12.2 Purpose

Learning goals define the intended educational outcomes for a diagnosis.

They establish what cases and education should collectively teach.

Learning goals support:

- case design
 - case coverage assessment
 - content generation
 - review
 - curriculum analysis
 - recall design
 - publication readiness
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12.3 Quality requirements

A learning goal should:

- describe an observable clinical reasoning outcome
 - be specific to the diagnosis
 - be teachable through Wardle content
 - avoid vague educational verbs
 - identify meaningful learner capability
 - remain stable enough for longitudinal coverage analysis
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12.4 Coverage

A diagnosis should not be considered educationally complete merely because learning goals exist.

Coverage should identify:

- which cases support the goal
- how strongly each case supports it
- which discriminators are covered
- which mimics are covered
- which requirements remain missing

Coverage is an assessment of implementation.

It is not the learning goal itself.

13. Diagnosis-Level Readiness

13.1 Purpose

Diagnosis-Level Readiness interprets whether the diagnosis foundation is sufficient for a defined next step.

Readiness must always be qualified.

Examples include:

- ready for scaffold generation
- ready for education generation
- ready for case generation
- ready for coordinated review
- ready for publication assessment

There is no single universal diagnosis-ready state.

13.2 Metadata readiness

A diagnosis is metadata-ready when its identity and required classification are sufficiently complete for downstream editorial development.

Metadata readiness may require:

- valid canonical identity
- resolved duplicate risk
- appropriate specialty
- appropriate taxonomy
- difficulty band
- operational eligibility

- reviewed playability
 - reviewed generatability
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13.3 Scaffold readiness

A diagnosis is scaffold-ready when enough diagnosis context exists to generate or author its educational foundation.

This may require:

- metadata readiness
 - clear educational scope
 - no unresolved identity conflict
 - generation eligibility where AI is used
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13.4 Case-generation readiness

A diagnosis is ready for case generation when the required educational and reasoning foundation exists.

Depending on policy, this may require:

- approved or reviewable editorial brief
- defined learning goals
- required teaching rules
- required mimics
- generation guidance
- sufficient reasoning support
- no blocking identity issue

Case-generation readiness authorises draft creation.

It does not authorise case approval or publication.

13.5 Education-generation readiness

A diagnosis is ready for education generation when its educational intent and clinical scope are sufficiently defined.

This may require:

- registry identity
- metadata

- editorial brief
 - teaching rules
 - required differentials
 - relevant investigation and management anchors
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13.6 Ready for diagnosis review

A diagnosis is ready for coordinated review when the minimum required artifacts exist in reviewable form.

This may include:

- registry metadata
- editorial brief
- teaching rules
- diagnosis education
- at least one case or justified absence
- identified differentials
- relevant reasoning and graph context

Generated but unreviewed content may contribute to review readiness.

It must not contribute to approval.

13.7 Onboarding completion

Onboarding completion indicates that the diagnosis has established its required initial editorial foundation.

Completion policy should be explicit and configurable.

It should not be inferred merely from the presence of records.

Rejected, incomplete, or placeholder artifacts should not satisfy completion requirements.

14. Diagnosis Governance Decisions

Diagnosis Governance may produce decisions including:

- activate registry entry
- hide registry entry
- deprecate registry entry
- approve metadata
- request metadata changes

- confirm playability
- restrict playability
- confirm generatability
- restrict generatability
- approve alias
- reject alias
- merge duplicate diagnoses
- approve editorial brief
- request brief revision
- activate teaching rule
- deprecate teaching rule
- mark onboarding complete
- reopen onboarding

Each decision should preserve:

- reviewer
- rationale
- affected version or state
- resulting status
- related diagnosis where relevant
- date and time

15. Duplicate and Merge Governance

15.1 Purpose

Duplicate diagnosis entries fragment knowledge and threaten editorial consistency.

Duplicate governance should consider:

- canonical name similarity
 - alias overlap
 - clinical equivalence
 - subtype distinctions
 - case mappings
 - education
 - graph relationships
 - teaching rules
 - historical use
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15.2 Merge principle

Diagnoses should only be merged where they represent the same clinical concept.

Similarity alone is insufficient.

The merge process should preserve:

- aliases
- case relationships
- education history
- differential mappings
- graph provenance
- teaching relationships
- audit history
- source identity

The merged diagnosis should remain historically traceable.

15.3 Non-merge examples

Diagnoses should not be merged where they differ meaningfully in:

- pathophysiology
- clinical presentation
- severity
- age group
- management
- educational reasoning
- accepted diagnostic criteria

Where related but distinct diagnoses exist, WEOS should use knowledge relationships rather than merging identity.

16. Diagnosis Maintenance

Diagnosis Governance continues after onboarding.

Maintenance may be triggered by:

- terminology changes
- revised classification
- duplicate detection

- guideline changes
- new educational priorities
- learner confusion
- graph inconsistency
- curriculum restructuring
- new case needs
- evidence changes

Maintenance may affect metadata, briefs, teaching rules, aliases, or operational permissions without requiring the diagnosis identity to be replaced.

17. Quality Standard for a Governed Diagnosis Foundation

A high-quality diagnosis foundation should be:

Clinically identifiable

The diagnosis is represented by a clear, accepted clinical identity.

Terminologically coherent

Canonical names, display labels, and aliases are deliberate and internally consistent.

Appropriately classified

Specialty, category, body system, difficulty, rarity, setting, age group, and urgency are meaningful.

Educationally scoped

The editorial brief defines what learners should understand and how content should teach it.

Governed by reusable rules

Teaching rules constrain and guide downstream content.

Operationally explicit

Playability, generatability, registry activity, and onboarding status are independently understood.

Relationship-ready

The diagnosis can participate safely in differentials, graph relationships, evidence relationships, and reasoning paths.

Continuously maintainable

Changes can occur without erasing identity or governance history.

18. Normative Statements

- Every diagnosis **MUST** possess a stable Diagnosis Registry identity.
 - Every playable case **MUST** resolve to a governed diagnosis identity.
 - Canonical diagnosis names **MUST NOT** be replaced by descriptive case titles.
 - Diagnosis aliases **MUST** represent clinically equivalent terminology.
 - Clinically distinct diagnoses **MUST NOT** be merged solely because their names or presentations are similar.
 - Registry activation **MUST NOT** be interpreted as editorial completion.
 - Playability **MUST** remain distinct from diagnosis activation.
 - Generatability **MUST** remain distinct from playability.
 - Automated generation **MUST NOT** occur for a diagnosis prohibited from generation.
 - Diagnosis metadata **SHOULD** describe the diagnosis rather than one specific case.
 - Learning goals **MUST** define meaningful educational outcomes.
 - Teaching rules **MUST** preserve stable identity and rationale.
 - The Editorial Brief **MUST** coordinate educational intent without replacing downstream artifacts.
 - Generated diagnosis artifacts **MUST NOT** count as approved merely because they exist.
 - Diagnosis readiness **MUST** identify the specific transition or operation for which the diagnosis is ready.
 - Onboarding completion **MUST NOT** replace artifact-level approval or publication governance.
 - Diagnosis merges **MUST** preserve provenance and historical relationships.
 - Diagnosis governance decisions **MUST** remain auditable.
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Closing Principle

A diagnosis is the enduring centre of Wardle’s editorial knowledge.

Before Wardle can safely generate, review, teach, or publish a diagnosis, it must first know exactly what the diagnosis is, how it should be classified, what learners should understand about it, and which rules should govern its educational expression.

Diagnosis Governance transforms a clinical label into a stable, purposeful, and governable foundation for diagnostic education.